ASCA
Ambulatory Surgery Center Association

June 16, 2025

Centers for Medicare & Medicaid Services
Department of Health and Human Services
ATTN: CMS-0042-NC
7500 Security Boulevard
Baltimore, MD 21244

To Whom it May Concern,

The Ambulatory Surgery Center Association (ASCA) submits the following comments in response to the Centers for Medicare & Medicaid Services (CMS), Assistant Secreaty for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology (ONC) Request for Information (RFI) on the Health Technology Ecosystem.

ASCA represents the interests of the more than 6,300 Medicare-certified ambulatory surgical centers (ASCs) nationwide. ASCs are located in every state and offer a high-quality, convenient and low-cost choice for patients who do not require hospitalization after surgical or diagnostic procedures.

Current State of Health IT in ASCs

While ASCs are an integral part of the nation's health care delivery system, they have not been incorporated in federal programs that encourage and incentivize the use of health information technology (health IT). Surgery centers did not receive any federal funding for EHR adoption in the HITECH Act of 2009. While ASTP/ONC estimates that at least 88 percent of office-based physicians[1] and 96 percent of acute care hospitals[2] had adopted an EHR by 2021, ASCA estimates that ***at most*** 50 percent of ASCs are using an EHR. Additionally, many of those ASCs with EHRs are likely using inpatient products that are ill-fitted to their operational needs.

Some aspects of low EHR uptake can be attributed to choice; ASCs tend to be small businesses that operate with a small staff and thin margins. Many centers simply do not have the financial means or staffing capability necessary for procurement, implementation and ongoing operation of an EHR. Conversely, even larger facilities with relatively more financial and/or staffing capabilities find greater implementation complexity due to the intricate nature of their needs. Surgery centers are also cognizant of ongoing costs related to cybersecurity, which is paramount for any health facility handling electronic health information. Finally, the core purpose of an ASC – to provide a single episode of care in less than 24 hours – does not require the ongoing coordination needed for treatment in an office or hospital.

[1] https://www.healthit.gov/data/quickstats/office-based-physician-electronic-health-record-adoption
[2] https://www.healthit.gov/data/quickstats/adoption-electronic-health-records-hospital-service-type-2019-2021

277 S Washington St Ste 375
Alexandria VA 22314-3646
Phone: 703.836.8808

asc@ascassociation.org
ascassociation.org

Recent surveys of ASCA members illuminate the potential difficulties facilities experience when incorporating EHRs into their operations. An April 2023 survey found that even surgery centers that use EHR have ongoing concerns about integration with external systems. Among surgery centers that reported use of an EHR, 34 percent stated concern regarding the ability to send and receive patient information and 40 percent stated concern regarding their ability to connect to external programs generally[3]. An August 2024 survey found that only 54 percent are currently using an EHR to facilitate reporting to the ASC Quality Reporting (ASCQR) program.

Both Congress and CMS have recognized the lack of EHR availability in ASCs. There is no federal requirement for ASCs to implement an EHR, and ASC-based clinicians (those clinicians who furnish 75 percent or more of their covered services in an ASC) are exempt from the Promoting Interoperability performance category of the Merit-based Incentive Payment System (MIPS). While ASCs are subject to policies in the 21st Century Cures Act Final Rule, the rule contains several exceptions for sites of service with limited access to electronically stored information, such as surgery centers.

Request for Information Responses

PR-8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers? In what ways can the interoperability and quality reporting responsibilities of providers be consolidated so investments can be dually purposed?

Aligning clinical quality data responsibilities with consistent quality measures within the ASC Quality Reporting (ASCQR) program will aid surgery centers in committing to data collection. Surgery centers are often small businesses with limited staff, and do not have the capability to devote even a single full-time employee to data collection and/or export. Additionally, recent fluctuations in the ASCQR measure set has created significant confusion from surgery center staff about their data collection responsibilities. Facilities may be more willing to commit to clinical data collection processes if they know that the quality measures will be stable (and thus worth the up-front investment in altering operations) and may also be used as a tool to show the relative quality of care compared to hospitals and offices.

PR-12: Should ASTP/ONC consider removing or revising any of the information blocking exceptions or conditions within the exceptions to further the access, exchange, and use of electronic health information (EHI) and to promote market competition?

ASCA strongly advocates that ASTP/ONC retain the Infeasibility and Manner exceptions with regards to information blocking. These exceptions provide relief pathways for provider types with limited access to health IT by allowing for fulfillment of requests for health information by alternative methods or by showing the incapability of fulfillment given the resources of the ASC. Revising either of these exceptions could create instances of undue burden for surgery centers and other provider types with limited health IT, and limit access to medical services for patients.

[3] ASCA 60-Second Survey, April 2023

Future Collaboration between ASTP/ONC and ASCA

ASCA staff has enjoyed a collaborative relationship with staff at ONC and continues to advocate for ASC-specific standards and maximum regulatory flexibilities. In 2018, ASCA staff presented at the ONC Annual Meeting[4] as part of ongoing work towards an ASC-specific EHR certification. However, ONC's movement away from certified EHR editions[5] made an ASC-specific certification less imperative. The federal exemptions above, coupled with ONC's sunsetting of certification editions meant that ASC-focused EHR vendors can now focus on creating products that best fit the needs of individual facilities rather than a set of federally mandated criteria. This is reflected in the fact that a majority of ASCs that do use an EHR use a product from a company without a certified EHR[6]. Surgery centers will undoubtedly be disadvantaged by a regulatory framework whose design relies on health IT (certified or otherwise) compared to their counterparts in hospitals and physician offices.

* * * * *

We value the Administrator Oz and ASTP/ONC's willingness to listen to our concerns as we strive to support our members in providing high-quality patient care. If you have any questions, please contact Kara Newbury at knewbury@ascassociation.org or 703.636.0705.

Sincerely,

Kara Newbury
Chief Advocacy Officer

[4] https://www.healthit.gov/sites/default/files/2019-01/ONC%20Annual%20Meeting%202018%20Slides_ASCA.pdf
[5] https://legacy.himss.org/resources/hti-1-final-rule-five-key-resources-understanding-new-standards-specifications
[6] ASCA 60-Second Survey, April 2023: 57 percent of ASCs with an EHR use either an SIS or HST Pathways product